

Our Ref: (T) TS/AS/ [MED-ID]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

[PROVIDER-NAME]

Joint IBD Clinic

[DATE]

Clinic Letter [DATE]

[PROVIDER-NAME]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[ADDRESS]

[POSTCODE]

Division B

Care Group - Emergency & Specialist Medicine Gastroenterology

Room CG89, Mailpoint [ADDRESS]

[ADDRESS]

[POSTCODE]

Telephone: [PHONE]

Fax: [PHONE]

Dear [PROVIDER-NAME] [NAME]

[ADDRESS]. [POSTCODE]

DOB: [DATE] , Hospital Number: [MED-ID] , NHS Number: [NHS-NO]

Diagnoses:

Mild Crohn's disease with no active treatment

Vitamin B12 deficiency leading to a peripheral sensory neuropathy

[NAME] canal stenosis C5/C6

I reviewed Mr [NAME]. From a GI perspective he is well. He opens his bowels once a day passing stool. He describes some abdominal cramps but this is usually in relation to particular foods always found a low-FODMAP diet helpful. He is also going to try some probiotics.

He continues to have vitamin B12 injections and more recently he has been having these every weeks. I suggested that he try to extend this period to every four to six weeks. Previous attempts at many people to try and reduce the frequency further have led to worsening symptoms although I think I have ever been convinced that this is necessarily related to vitamin B12 concentration. His recent bloods were all normal. I have asked him to submit a stool sample for faecal calprotectin as part of the monitoring process for his Crohn's disease and I will write when the results are available. We will otherwise see him in a year's time.

With best wishes

Yours sincerely

[NAME]

[NAME]

[DICTATION-META]

[PROVIDER-NAME]

Consultant Gastroenterologist